This document is a Request for Information (RFI). It does not constitute a solicitation, a commitment to issue a solicitation, or an obligation to award a contract. Responses will be used for market research purposes only.

Request for Information (RFI)

SI-Assisted Omni Channel Experience

Centers for Medicare & Medicaid Services

Notice ID: 279999
Issued: November 13, 2026
Response Deadline: January 5, 2027
Issuing Office: CMS Office of Acquisition and Grants Management (OAGM)
Program Office: CMS Office of Communications (OC)

Section 1: Purpose and Background

1.1 Purpose

The Centers for Medicare & Medicaid Services (CMS) operates one of the largest contact center ecosystems in the federal government, serving people with Medicare, ACA Marketplace consumers, and caregivers across phone, chat, web, email, and mail channels.

CMS is conducting market research to inform a potential future acquisition for SI-assisted omni channel experience capabilities. This updated RFI builds on an initial market research round (Notice ID 269999, February 2026), its scope is being expanded to include the ACA Marketplace, and incorporates findings from commercial payer benchmarking, vendor landscape analysis, and an internal Discovery Sprint process.

This RFI is issued solely for market research and information purposes. It does not constitute a solicitation, a commitment to issue a solicitation, or an obligation to award a contract. No proposals are being requested or accepted in response to this RFI. CMS will not pay for information provided in response to this RFI.

1.2 Background

CMS operates one of the largest government contact center ecosystems in the United States, serving people with Medicare and ACA Marketplace consumers across phone, chat, web, email, and mail

channels.

CMS's Vision & Strategy 2029 establishes a phased transformation from a staffing-first contact center model to an SI-first, omnichannel platform built on common infrastructure and technologies. Rather than a single turnkey operation, this ecosystem is designed to support distinct programs with different populations and demands, drawing on best-of-breed capabilities configured to each program's needs:

Medicare Program — Serving approximately 67 million Americans managing ongoing coverage, premium inquiries, benefit utilization, care navigation, and fraud reporting.

ACA Marketplace Program — Managing 21 million enrollees, with extreme seasonal demand (3–4x volume spikes during Open Enrollment Periods), , enrollment notices, and tax/form complexity.

1.3 Purpose of RFI

This updated RFI serves three purposes:

1. **Validate and deepen vendor capability data.** This RFI continues market research, building on information received from the previous RFI (Notice ID 269999, February 2026) and incorporating findings from commercial payer benchmarking and vendor landscape analysis. CMS is seeking evidence-based updates on vendor capabilities, pricing models, implementation approaches, governance and evaluation frameworks, and workforce transition approaches. CMS is particularly interested in responses that demonstrate sustained, production-validated performance, not launch-period projections.

2. **Surface evidence that resolves CMS's open Discovery Sprint questions,** including the boundary between SI-assisted and human-required interactions, the evaluation and go/no-go criteria for scaling pilots to production, and the workforce transition and change management approaches required as containment increases. These questions are detailed in Section 5.

3. **Inform a defined downstream process**. Vendor responses will directly feed the CMS Discovery Sprint, a structured four-week internal process, and the subsequent acquisition strategy. Responses will not be evaluated as proposals and do not obligate CMS to any acquisition path.

1.4 Discovery Sprint Context

CMS is conducting a structured Discovery Sprint to validate assumptions about consumer needs, SI deployment readiness, before committing to a procurement direction.
Vendor responses to this RFI will directly inform subsequent procurement planning.

Section 2: Conflict of Interest Disclosure Requirements

2.1 Mandatory COI Certification

All respondents must complete and submit the following certification with their RFI response. Failure to provide this certification will result in the response being disqualified from consideration.

By submitting a response to this RFI, the respondent certifies that:

1. The respondent is **not** an insurance carrier or health plan offering Medicare, Medicaid, or Marketplace products.
2. The respondent is **not** an insurance producing agency or brokerage that earns commissions from Medicare, Medicaid, or Marketplace plan sales.
3. The respondent is **not** a Field Marketing Organization (FMO) whose business model relies on insurance sales commissions.
4. The respondent is **not** an ACA Marketplace Enhanced Direct Enrollment Entity or Direct Enrollment Entity
5. The respondent does **not** have a financial incentive to steer Medicare or Medicaid beneficiaries or Marketplace consumers toward specific plans or carriers.
6. The respondent does **not** have an ownership, affiliation, or revenue-sharing relationship with any entity described in items 1–5 above.

If any of the above conditions apply, the respondent must disclose the nature of the relationship and describe proposed mitigation measures. CMS will consider disclosed conflicts of interest, and the adequacy of proposed mitigation, in determining whether a respondent's input is used to inform market research and any future acquisition strategy. Failure to disclose a known conflict of interest may affect CMS's consideration of that response in future solicitation planning.

2.2 Disclosure Questions

COI-1. Does your organization have any current or recent (within the past three years) ownership, affiliation, partnership, subcontracting, or revenue-sharing relationship with any insurance carrier, health plan, insurance agency, brokerage, or FMO? If yes, please describe the relationship and explain how your organization would mitigate any conflict of interest in performing work for CMS.

COI-2. Does any parent company, subsidiary, or affiliated entity of your organization earn revenue from Medicare, Medicaid, or Marketplace insurance sales, commissions, or referral fees? If yes, please describe the relationship and proposed mitigation.

COI-3. Has your organization previously responded to CMS RFI Notice ID 269999? If yes, please confirm whether your COI status has changed since that submission.

Section 3: Vendor Profile and Qualifications

3.1 Organization Information

ORG-1. Provide your organization's legal name, DUNS/UEI number, CAGE code, business size, and applicable NAICS codes.

ORG-2. Describe your organization's primary business focus and core competencies as they relate to SI-assisted contact center modernization.

ORG-3. Describe your organization's experience delivering SI-enabled contact center solutions to federal agencies. For each relevant engagement, provide agency name (or anonymized description if required), contract vehicle, period of performance, scope of services, number of contacts handled annually, and key performance outcomes. Distinguish between prime contractor and subcontractor roles.

ORG-4. Describe your organization's technology experience supporting contact center operations and/or technologies at the scale of CMS (multiple lines of business with interaction volumes greater than 40M annually). Please address the following:

- The largest contact comparable contact center modernization deployment your organization has supported. Please include by annual interaction volume, and how that scale compares to CMS's contact center operations.
- Whether your organization served as prime contractor, subcontractor, or technology partner.
- If your role was part of a team: identify which organization owned, licensed or subcontracted the core SI/agentic technology that was used, and describe the nature of that relationship (proprietary build, commercial license, OEM/white-label, or subcontracted development)
- How your organization's experience – whether as an integrator or a specialized SI vendor – translates to CMS's requirement, even if your direct experience is at smaller scale but in a highly comparable domain (e.g., health benefits navigation, regulated consumer contact centers, high-stake eligibility support)

ORG-5 Identify how you would most likely team to meet CMS requirements (i.e.; teaming arrangements, subcontractors, or technology partners). For each, describe their role, their COI status under Section 2, and how the team's combined capabilities address CMS's requirements. Specifically address:

- Ownership of core SI/agentic technology. Identify which organization on the team owns, develops, licenses or, subcontracts the SI/agentic capability being proposed (e.g., proprietary build, commercial license, OEM/white-label, or subcontracted development).
- If responding as a generalist integrator or prime: describe how you selected your SI technology partner (s), what alternatives you considered, and how you evaluated and maintain accountability for a subcontracted SI vendor's performance, compliance, and equity outcomes.
- If responding as a specialized/niche SI vendor: describe whether you intend to respond as a prime, as a subcontractor to an integrator, or both, and identify any integration, scale, or contract-vehicle gaps you would need a teammate to fill.
- How the team's structure affects CMS's visibility into the technology actually performing beneficiary or consumer interactions, and what contractual or reporting mechanisms would preserve visibility.

ORG-6 Describe any experience supporting health insurance, Medicare, Medicaid, or ACA Marketplace contact center operations, either as direct provider of the service or as the underlying SI/technology partner supporting another organization's delivery of service. Address the following:

- Populations served, contact types handled, and any equity or access considerations addressed (e.g., language access, disability accommodation, digital literacy, low-bandwidth or non-digital channels).
- Whether experience was delivered directly, as a subcontractor, or as a licensed technology embedded in another vendor's solution

- If your direct experience is an adjacent regulated domain (e.g., other public benefits programs, regulated financial services, or high-stakes consumer eligibility decisions) rather than health insurance specifically, describe how that experience transfers to CMS's requirements.
- Any evidence of your SI/technology performance specifically, as distinct from the performance of an organization that deployed it, such as containment rates, accuracy metrics, or equity outcomes attributable to your underlying technology.

ORG-7 Describe the CMS-side staffing, oversight, and technical resourcing your proposed engagement model would require — for example, dedicated program management, data stewardship, integration engineering, or content/knowledge ownership roles. For a comparable federal or large-enterprise engagement, what internal capacity did the client organization need to provide, and what happened in cases where that capacity was insufficient? What is the minimum viable CMS-side team you would recommend to avoid delivery risk?

ORG-8 Distinguish CMS-side resource needs attributable to the core SI/agentic technology itself versus resource needs attributable to integration, program management, or surrounding systems. If your proposed engagement model relies on teaming or subcontracting (per ORG-G), describe how CMS-side resource needs would change under that structure compared to a single-vendor delivery model.

Section 4: Technical Capabilities

4.1 Agentic SI and Conversational SI

TECH-1. Describe your organization's agentic SI and conversational SI capabilities for contact center applications.

- How does your solution handle multi-step, multi-intent consumer interaction? Provide examples of containment rates achieved in comparable deployments, including the domain, scale and channel(s) involved.
- Is the agentic SI/conversational SI engine proprietary to your organization, commercially licensed, or a subcontracted/OEM from another provider? If not proprietary, identify the source and describe your organization's role in configuring, training, or operating it.

If your comparable deployment experience is at a smaller scale than CMS's, describe how your containment and multi-intent handling results would be expected to scale, and what risks or unknowns would remain unproven at CMS's volume.

TECH-2. How does your SI solution handle interactions that exceed its resolution capability?

- Escalation logic. What criterion and mechanisms does your SI use to determine when a human handoff is required, including both automated triggers (confidence thresholds, intent classification failures, regulatory/safety flags) and consumer-initiated escalation requests?
- Warm transfer and routing. Describe your approach to transferring the interaction to a live agent without requiring the caller to repeat themselves. Identify whether this capability is native to your solution, dependent on a specific telephony platform, or requires integration with a third-party routing system.
- Context preservation. Describe how interaction history, caller intent, and prior authentication are passed to the receiving agent at handoff. Identify whether this capability is native to your solution or dependent on an external RM or case management platform. If the latter, which platforms you have built and tested integrations with.

- If your solution handles escalation logic only: describe how you have partnered with or integrated into telephony and CRM platforms to deliver end-to-end handoff capability, and what gaps or dependencies CMS should be aware of.

TECH-3 Describe your organization's approach to scaling from pilot to production in a large federal contact center environment. What are the key success factors and common failure modes?

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- Was your organization responsible for the SI/agentic technology itself, the surrounding program/integration management, or both, in comparable pilot-to-production transitions? If your role was primarily technology or primarily program management, described how you would partner with a complementary organization to cover the other function at CMS's scale.
- If your direct experience is at a smaller scale than CMS's production volume, describe your basis for confidence that success would carry over, and any risks that remain unproven at this scale.

TECH-4. Describe your solution's approach to intent recognition and resolution for complex, policy-dependent How does your solution stay current with policy changes?

TECH-5. How does your solution support proactive outreach? Describe any experience with outbound SI-assisted communications in a government or healthcare context.

4.2 Omnichannel Integration/Platform and Infrastructure Integration

Note: CMS is separately modernizing its CRM/case management platform, which is expected to serve as the system of record for consumer interaction history and cross-channel context.

TECH-6. Describe your solution's omnichannel capabilities across phone (IVR/IVA), chat, web, email, and mail channels. Address the following:

- How does your solution consume and pass context (e.g., prior interactions, intent, authentication status) to and from an external CRM/case management system, rather than maintaining that context independently?
- What data or event triggers does your solution requirement from a CRM to preserve continuity as a beneficiary or consumer moves across channels?
- Identity any context or continuity capability your solution provides natively, distinct from what you would expect the CRM to provide, an explain why that capability should remain with your solution rather than the CRM.
- Describe integration methods (APIs, webhooks, event streaming) your solution has used to exchange interaction context with CRM platforms in comparable deployments.

TECH-7. Describe your experience integrating SI-assisted contact center solutions with existing CRM platforms, cloud contact center infrastructure (e.g., Amazon Connect, Genesys, Cisco), and government data systems.

TECH-8. Describe your solution's cloud-native capabilities and your experience supporting cloud migrations in active contact center environments without service disruption.

4.3 SI Knowledge Management

TECH-9 Describe your approach to knowledge base architecture, content governance, and ongoing knowledge quality management. Address:

- How is the knowledge base structured and maintained?
- Who owns knowledge content — the vendor, the agency, or a shared model?
- How does your solution perform when underlying knowledge is incomplete, outdated, or inconsistent?
- What quality metrics do you track for knowledge content?

Provide examples of knowledge remediation work performed as part of prior deployments, including the scope of the remediation and measurable outcomes.

TECH-10 Describe how your solution identifies knowledge gaps — situations where the SI cannot answer a question because the knowledge base is insufficient. How are these gaps surfaced to content owners? What is your process for prioritizing and closing knowledge gaps? Provide data on average knowledge gap closure timelines from a production deployment.

TECH-11 Provide your current FedRAMP authorization status for each component of your proposed solution:

- FedRAMP In Process (provide Package ID and estimated authorization date)
- Agency ATO (provide authorizing agency, authorization date, and expiration date)
- JAB Provisional ATO (provide authorization date and impact level)
- Not yet authorized (provide your path to authorization and realistic timeline)

Identify any third-party components (SI models, APIs, data services) that are not yet FedRAMP authorized and describe how you will address this gap. CMS requires a realistic, dependency-mapped ATO timeline — not a best-case estimate.

TECH-12 Describe how your solution handles Protected Health Information (PHI) in compliance with HIPAA. Address: data minimization practices, PHI retention and deletion policies, audit logging, breach notification procedures, and any limitations on SI model training using CMS beneficiary data. Describe any constraints your PHI/HIPAA compliance posture places on SI personalization capabilities.

Section 5: Discovery Sprint Support Questions

This section is specifically designed to solicit vendor input that will directly inform CMS's four-week Discovery Sprint. CMS is particularly interested in vendor perspectives on the open questions the sprint is designed to answer. Detailed, evidence-based responses to this section are strongly encouraged.

5.1 Problem Definition and Scope

SPRINT-1. CMS serves two distinct program populations; Medicare beneficiaries (predominantly 65+, many with limited digital access) and ACA Marketplace consumers (broader age range, higher digital literacy, extreme seasonal volatility). In your experience, do these populations require fundamentally different SI deployment strategies, or can a common platform serve both with appropriate configuration? What are the key differences in approach?

SPRINT-2. Contact friction for CMS customer's is primarily **informational** (beneficiaries/consumers don't have the information they need), **navigational** (beneficiaries/consumers can't find or access the right channel), or **trust-based** (beneficiaries/consumers don't trust digital or SI-assisted channels). In your experience with comparable populations, which of these friction types is most prevalent, and how does your solution address each?

SPRINT-3. In your experience, how do policy changes affect contact volume patterns, and how quickly do the effects materialize in call data? Provide examples of how your technology solution adopted quickly to handle call volumes and patterns.

5.2 SI Deployment Boundaries and Governance

SPRINT-4. Describe your organization's approach to defining the boundary between SI-assisted interactions and human-required interactions in a government benefits context. What criteria do you use to determine when an interaction is too complex, too high-stakes, or too sensitive for SI resolution? How is this boundary maintained and updated over time?

SPRINT-5. Describe the evaluation framework your organization would recommend for assessing pilot performance, including metrics for containment, CSAT, compliance, equity, and beneficiary safety. What go/no-go criteria would you recommend for scaling a pilot to production?

SPRINT-5A. Describe how your recommended evaluation framework (per SPRINT-7) would be governed on an ongoing basis after initial pilot approval — including cadence of re-evaluation, ownership of go/no-go decisions between CMS and vendor, and process for revoking or scaling back SI authority for a use case if performance degrades in production. What early-warning indicators should trigger a governance review before a formal go/no-go checkpoint?

SPRINT-6. What is your organization's recommended approach to **separate CSAT measurement** for SI-assisted vs. live-agent interactions? How should CMS structure its measurement framework to ensure that efficiency gains do not mask declining beneficiary satisfaction in SI-handled interactions?

5.3 Health Equity and Access

SPRINT-7. CMS serves diverse populations with digital access and literacy gaps. Describe your solution's specific capabilities for serving diverse populations. How does your solution ensure that SI-assisted containment improvements do not inadvertently reduce access for the populations that most need support?

SPRINT-8. Describe your organization's approach to **equity measurement** in SI-assisted contact center operations. What metrics do you recommend for tracking equity outcomes across beneficiary populations? How do you identify and remediate SI performance disparities across demographic groups?

SPRINT-9. How does your solution support beneficiaries who prefer or require human interaction — either by choice or due to the complexity of their situation, even when SI resolution is technically possible? Describe your approach to ensuring that the human option remains accessible and not artificially difficult to reach.

SPRINT-10. Describe your experience supporting **multilingual** contact center operations, including real-time translation, LEP-specific IVR/IVA flows, and agent support for non-English interactions. What languages does your solution currently support, and what is the process for adding new language support?

5.4 Workforce Transition

SPRINT-11. SI-first contact center transformation will reduce demand for traditional agent roles over time. Describe your organization's experience supporting **workforce transition** in large contact center environments, including reskilling programs, role redesign, and change management. What are the most common workforce transition challenges, and how have you addressed them?

SPRINT-12. As SI containment increases, the residual interactions handled by human agents will become progressively more complex, sensitive, and high-stakes. Describe your approach to **agent augmentation** — tools, real-time guidance, and knowledge management — that help agents handle this more complex workload effectively. How does your solution reduce Average Handle Time (AHT) for complex interactions without sacrificing quality?

SPRINT-13. Describe your organization's approach to **change management** for contact center modernization in a large federal agency environment. What stakeholder engagement, communication, and training strategies have been most effective in comparable engagements?

5.5 PHI, HIPAA, and Data Governance

SPRINT-14. Describe your solution's approach to **PHI/HIPAA compliance** in SI-assisted contact center operations. How does your solution handle the collection, processing, storage, and transmission of protected health information? What certifications, audits, and compliance frameworks does your solution maintain?

SPRINT-15. CMS is exploring SI personalization capabilities using beneficiary and consumer data to tailor interactions and proactively surface relevant information. Describe the **PHI/HIPAA constraints** that apply to SI personalization in a Medicare/Medicaid or Marketplace context, and how your solution navigates these constraints. What is feasible today, and what requires additional policy or regulatory clarity?

SPRINT-16. Describe your solution's **data governance** capabilities, including data lineage, audit logging, access controls, and retention policies. How does your solution support CMS's compliance auditability requirements for SI-assisted interactions?

5.6 Sprint Planning and Collaboration

SPRINT-17. CMS's Discovery Sprint is structured across four weeks: Week 1 (Baseline), Week 2

(Discovery), Week 3 (Analysis), and Week 4 (Synthesis). If your organization were engaged to support this sprint, what specific contributions could you make to each phase? What data, tools, or expertise would you bring to accelerate the sprint's findings?

SPRINT-18. The sprint will produce pilot scorecards, go/no-go criteria, and a refined FY2026–2028 roadmap. What **implementation risks** do you believe CMS should prioritize in its go/no-go criteria? What are the most common reasons SI contact center pilots fail to scale to production, and how should CMS structure its criteria to avoid these failure modes?

SPRINT-19 The ACA Marketplace experiences 3–4x contact volume spikes during Open Enrollment Periods. Describe how your solution scales to manage OEP surge demand. Address: elastic capacity provisioning, surge-specific containment strategies, staffing model integration, and how your solution maintains quality and equity standards during peak periods. Provide data on OEP or comparable surge performance from prior deployments.

SPRINT-20 Describe how your pricing model accommodates seasonal demand variability. Can your solution support pricing by resolved contact or contained interaction rather than by staffed hour? What risk-sharing arrangements have you executed with federal or commercial clients to align vendor incentives with CMS outcomes?

Section 6: Pricing and Acquisition Strategy

PRICE –0 – In your responses to PRICE-1 through PRICE-6, please specify which layer(s) of the solution your pricing response covers (e.g., "core SI/agentic technology only" "full-stack including integration and support", or "technology component within a proposed team, pricing separately from the prime's integration cost."

PRICE-1. Describe your pricing model for the proposed solution. Provide indicative pricing ranges for: per-agent-seat licensing, per-interaction pricing, platform/infrastructure fees, implementation and integration costs, and ongoing support and maintenance. Identify which cost elements are fixed versus variable. What pricing model do you recommend for a large federal contact center modernization engagement, and why?

PRICE-2. Provide a total cost of ownership (TCO) estimate for a deployment of your solution at CMS's scale (approximately 30–40 million contacts annually across Medicare and Marketplace). Include: implementation costs, integration costs, licensing/subscription fees, knowledge management and content costs, training costs, and ongoing support costs. Identify cost elements that are typically underestimated in comparable deployments.

PRICE-3. Describe your firm's ability to support outcome-based or performance-based pricing models, including:

- ☐ Pricing per resolved contact or per contained interaction
- ☐ Pricing per verified CSAT outcome
- ☐ Shared savings arrangements tied to measurable cost reduction
- ☐ Risk-sharing constructs that align vendor incentives with CMS outcomes

What outcome-based or risk-sharing arrangements have you executed with federal or commercial clients? Provide specific examples including the metrics used, the risk/reward structure, and the outcomes achieved

PRICE-4. Describe the cost implications of workforce transition associated with your proposed solution, including:

6.1 Estimated costs for reskilling or upskilling existing contact center staff into augmented-agent or oversight roles
6.2 Severance, redeployment, or contract-labor adjustment costs typically incurred during comparable transitions
6.3 "Dual-running" costs — i.e., costs incurred while legacy staffing and new SI-enabled capacity operate in parallel during a phased rollout

For a comparable federal or large-enterprise engagement, provide actual or representative figures for workforce transition costs as a percentage of total contract value, and describe how these costs were budgeted, phased, and by whom (vendor vs. client) they were borne.

Section 7: Response Instructions

7.1 Format and Length

Deadline: Responses must be submitted by January 5, 2027 at 2:00 PM Eastern Time.

Submission Method: Vendors must submit responses by uploading their files to the designated CMS Box folder:

https://cmsbox.app.box.com/f/2aa029c164ae47509a35879ffde9e2c3

Email and SAM.gov submissions will not be accepted.

Response Format — Questions: Vendors must submit their responses to the questions in Sections 2 through 6 using the RFI Vendor Response Spreadsheet provided with this RFI. Enter each response in the row and column assigned to that question. Do not retype questions into a narrative document in place of completing the spreadsheet.

Supplemental Documents (Optional): Vendors may also upload supplemental documents to provide additional context, evidence, or supporting detail beyond what the spreadsheet captures. Supplemental documents are optional and limited to 40 pages combined, excluding organizational charts, and appendices containing production performance data.

Supplemental Document Format: PDF or Microsoft Word. Font: Times New Roman or Arial, 11-point minimum. Margins: 1 inch minimum.

7.2 Organization

- **RFI Vendor Response Spreadsheet** (completed, covering base question responses to Sections 2 through 6)
- **Supplemental documents**, if submitted, organized to mirror the section structure of this RFI, with each supplemental response clearly identifying the question number it addresses (e.g., TECH-1, SPRINT-5). For Section 5, respondents may answer all questions or select the subset most relevant to their proposed solution — please indicate which questions you are not answering and why.
- **Appendices**, if applicable: production performance data, system architecture diagrams, sample reports, and other supporting materials

7.3 Proprietary Information

Respondents should clearly mark any proprietary or confidential information. CMS will handle marked information in accordance with applicable law and regulation. However, respondents are encouraged to provide as much non-proprietary information as possible to support CMS's market research.

7.4 Submission

Responses — the completed RFI Vendor Response Spreadsheet and any optional supplemental documents — must be uploaded to the CMS Box folder at https://cmsbox.app.box.com/f/2aa029c164ae47509a35879ffde9e2c3 by January 5, 2027 at 2 PM EASTERN TIME.

Section 8: Disclaimer

This RFI is issued solely for market research and information purposes. It does not constitute a solicitation, a commitment to issue a solicitation, or an obligation to award a contract. CMS is not seeking proposals and will not accept unsolicited proposals in response to this RFI. CMS will not pay for information provided in response to this RFI. Respondents are advised that CMS is under no obligation to acknowledge receipt of the information received or provide feedback to respondents with respect to any information submitted.

The issuance of this RFI does not restrict CMS to any particular acquisition strategy. CMS reserves the right to cancel this market research at any time without obligation. Responses may be shared with CMS's Discovery Sprint team and authorized support contractors under appropriate non-disclosure protections. CMS reserves the right to contact respondents for clarification following submission.
